

NEW

2026

Ethiopian Health Profession Licensing Examination(EHPLE)

INFORMATION BOOKLET

PUBLIC HEALTH



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MINISTRY OF HEALTH-ETHIOPIA

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HEALTHIER CITIZENS FOR PROSPEROUS NATION



AAU-Testing Center

Message From the State Minister, Ministry of Health -Ethiopia



H.E Mrs. Frehiwot Abebe

Improving healthcare quality is a global priority for sustainable development, with high quality healthcare being a key component of universal health coverage. One strategy to maintain health care standards is through provision of health professionals' competency assessment. Consequently, in 2019, the Ministry of Health-Ethiopia, initiated the Ethiopian Health Profession Licensing Examination (EHPLE) for undergraduates in seven health disciplines, which has since expanded to include 17 health disciplines.

The main goal of this competency assessment is to identify health professionals with minimal competencies necessary to perform their duties safely and competently, thus enhancing the quality of health care services. This initiative is overseen by a dedicated Health and Health Related Institutions and Professionals' Regulatory Lead Executive Office (LEO), comprising four desks, which plays a pivotal role in strengthening the system and enabling the LEO to conduct the competency exam more extensively and with improved organization and quality.

It is important to note that this competency assessment differs significantly from traditional academic or employment examinations. Hence, this information booklet has been created to address the informational needs of both examinees and teaching faculty regarding the EHPLE. Additionally, it aims to facilitate the assessment process, while promoting transparency and ensuring the sustainability of the program.

The preparation of this booklet involved the collaboration of esteemed experts from various higher education institutions, AAU-IER, the Ministry of Health, JHPIEGO-Ethiopia, Amref, Health Professionals 'Associations, and the Ministry of Education. Their invaluable contributions are acknowledged with sincere gratitude, alongside appreciation for the Ministry of Health staff for their unwavering commitment and hard work throughout the project.

Acknowledgements

This Information Booklet for EHPLE is a contribution from several educators, researchers, students and concerned individuals with a genuine interest to propel Ethiopia's medical and health sciences education forward.

The Ministry of Health is grateful for the contribution of many individuals and institutions in realizing this endeavor. Among these are Professional Associations, Student Association, Higher Education Institutions (both public and private), JHPIEGO-Ethiopia, AMREF/HWIP, MoE (Ethernet), UNFPA, AAU-IER and all HHRIPR LEOs staff.



Acronyms and Abbreviations

EHPLE	Ethiopian Health Profession Licensing Examination
ETA	Educational and Training Authority
HEIs	Higher Education Institutions
HHrIPR-LEO	Health and Health-related Institutions and Professionals Regulatory Lead Executive Office
HSTP-II	Health Sector Transformational Plan-II
MCQ	Multiple Choice Question
MOH	Ministry of Health
WHO	World Health Organization



Purpose of the Information Booklet

The EHPLE Information Booklet serves as a comprehensive guide for those individuals seeking information about the exam. It typically outlines basic information for candidate registration, exam development and administration processes and procedures, result notification, and the licensing process. It also includes information on the exam framework, i.e., the exam domain, sub-domain, content, process, and task, with sample exam items specific to each profession.

The publication of this Booklet is crucial for the following reasons:

- **Clarity and guidance:** It provides clear information about the exam by ensuring candidates understand the necessary information to prepare them.
- **Accessibility:** It serves as a readily accessible resource for individuals pursuing to take the exam, consolidating essential information in one document and facilitating easy access to necessary details. It also helps other stakeholders who might be interested in such resources.
- **Transparency:** It promotes transparency in the examination process and fosters trust among stakeholders about the exam.

In summary, the publication of this Booklet is essential for creating a transparent, standardized, and accessible framework that guides candidates through the EHPLE process.

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Definition of terms

- **Domain:** a broad category or area of knowledge or skills of a profession
- **Sub-domain:** a subset of a broader domain that focuses on knowledge or skills related to the overarching domain
- **Content:** a more specific subcategory, which is a breakdown of the sub-domain
- **Task:** the responsibility, knowledge, skill, and attitude of a junior undergraduate professional in an actual work environment
- **Process:** a systematic sequence of steps or actions designed to achieve a specific outcome
- **Learning outcome:** a clear and measurable statement that describes what the examinee is expected to know or be able to do
- **Relative emphasis:** the proportional importance or weight assigned to different content areas or categories within the assessment
- **Item:** a particular multiple-choice question
- **Item developer:** a subject matter expert responsible for writing test items or questions that make up the examination
- **Item reviewer:** a subject matter expert responsible for reviewing and refining the test items or questions that make up the examination
- **Standard setting:** a process of determining a cut-off point or passing score for an exam
- **Item difficulty index:** a statistical measure that indicates the proportion of examinees who answered a particular test item correctly
- **Discrimination index:** a statistical measure that evaluates how well a particular test item differentiates between high-performing and low-performing examinees
- **Admission paper:** a printout paper generated by the system after completing registration that contains the examinee's photo, QR code, and necessary information

1. Introduction

1.1. Background

Competency assessment is one of the strategies for controlling the standard of healthcare services provided in healthcare facilities. The World Health Organization (WHO) recommends all healthcare professionals to have necessary competencies. In Ethiopia, the Health Sector Transformational Plan-II (HSTP-II) states competency assessment of all graduates before joining the health workforce as one of the strategic initiatives.

The Ministry of Health (MOH) launched the Ethiopian Health Profession Licensing Examination (EHPLE) for undergraduates in 2019. The Health and Health-related Institutions and Professionals Regulatory Lead Executive Office (HHrIPR-LEO) of the Ministry of Health is given a mission to implement the Ministry's objective to achieve competency-related goals. It has the responsibility to ensure that the EHPLE meets technical, professional, and legal standards, and to protect the health, safety, and welfare of the public by assessing candidates' abilities to practice competently.

Currently, the exam is given for 17 health professions (Medicine, Nursing, Public Health, Pharmacy, Medical Laboratory Science, Anesthesia, Midwifery, Dental Medicine, Medical Radiology Technology, Environmental Health, Psychiatric Nursing, Pediatric and Child Health Nursing, Emergency and Critical Care Nursing, Human Nutrition, Surgical Nursing, Optometry, and Physiotherapy). Since its introduction until December 2025, a total of 204,811 examinees took the exam in 23 rounds.

1.2. The Rationale of EHPLE

One of the critical functions of the MoH is to guarantee the efficiency, quality, and equity of healthcare delivery and to protect the public from any undesirable consequences in healthcare delivery practices. As professionals' competence is a significant determinant of the quality of health, evaluation of health professionals' competence has now been given due attention. The licensing examination for health professionals serves as a crucial step to ensure that individuals entering the field meet specific competency standards. The sole aim of the competency assessment is to safeguard public health by verifying that health professionals have the minimal basic knowledge, attitude, and skill required to provide safe and effective care.

Licensing exams act as a preventive measure, ensuring that only competent professionals join the health workforce, which, in turn, contributes to reducing the occurrence of medical errors and enhancing overall patient safety. By setting standards through examinations, regulatory bodies strive to minimize the risk of medical errors caused by incompetence.

2. Key processes of EHPLE

EHPLE involves several key processes to ensure the quality and reliability of the examination.

2.1. Registration of candidates

EHPLE has a mandatory online registration system for both new and repeat candidates, which can be found at <http://hple.moh.gov.et>

Please note these important notes during registration.

New Test Takers:



- The list of eligible candidates from governmental and private Higher Education Institutions (HEIs) will be uploaded to MoH's online registration system by the registrar office of their respective HEIs.
- Candidates who completed their education abroad must bring the following documents in person to MoH in order for their names to be uploaded to the online registration system:
 - ✓ Equivalence document from ETA
 - ✓ Externship attachment completion letter after completing the externship assigned by the regulatory body
- Once the name of the candidate is uploaded to the system and registration has opened for the current exam round, the candidate must register at <http://hple.moh.gov.et> by uploading the necessary documents listed below.
 - ✓ a scanned original or temporary degree
 - ✓ a scanned national ID

Repeat Test Takers:



- Since the information about re-exam candidates already exists in the system, the candidate should register by directly going to <http://hple.moh.gov.et>. There is no need for re-exam candidates to upload their documents.

Both new and repeat candidates:



- After completing the registration, the candidate must download and print the admission paper by logging into his/her account using username and password, or national ID FAN.
- The candidate can change the exam center by logging into his/her account only during the registration period
- An application for center change is not allowed after the registration period is closed.

2.2. Task Analysis

The first step of exam development involves conducting a comprehensive task analysis study, which identifies the tasks, knowledge, skills, and abilities required from a junior undergraduate professional in the specific profession. The analysis is typically done through surveys, interviews, or observations of practitioners in the actual work environment, as well as through the Delphi method with subject matter experts.

2.3. Exam Blueprint

Based on the task analysis findings, the exam blueprint is prepared that outlines the content areas to be covered in the examination and the weight or emphasis given to each area. This ensures that the exam reflects the key competencies and knowledge needed for competent practice in that specific profession. Blueprint is the matrix or chart that shows the number and type of test questions represented across the topics in the content area, consistent with the learning outcome and relative weight of the test given to each content area. The blueprint also identifies the percentage weighting of cognitive dimensions as the level of competence tested in each knowledge domain.

Key components of blueprint are:

- Domain
- Sub-domain
- Content
- Task
- Process
- Learning outcome
- Assessment methods
- Assessment tools/instrument (test format)
- Relative emphasis (in percentage)

2.4. Item Development

The items are developed following specific guidelines to ensure clarity, relevance, and fairness. Subject matter experts with experience in the field are selected from HEIs to develop exam questions (items) that align with the exam blueprint. The exam questions focus mainly on “knows how” according to the competency level of the Miller's pyramid. The items are produced in a secure location on designated computers that are free from internet connectivity. The items are scenario-based and constructed with stem, lead-in, and four options/alternatives. All items will have a single-best-answer type of Multiple-Choice Question (MCQ) that addresses the learning outcome defined in each content area. Standard textbooks, updated guidelines, and standards are used as reference materials.

2.5. Item Review

Once developed, the items undergo a rigorous review process by item reviewers. The main purpose of the exam review process is to evaluate content relevance, technical accuracy, clarity, and sensitivity related to culture and religion. More experienced subject matter experts as well as psychometric experts will do the review to ensure the items meet psychometric standards. Subject matter experts shall review the items to confirm that they are accurate, clearly stated, and correctly keyed using the checklist. Psychometric experts shall review the items to ensure that

they are not technically flawed. They also work on editorial review to check grammar, punctuation, and spelling errors. This helps ensure the reliability and validity of the items.

2.6. Standard setting method

The standard setting or cut-off point of the EHPLE is determined using the Modified-Angoff method, which is one of the most widely used and legally defensible standard setting approaches to set a cut-off point for high-stake competency examinations.

The method involves a panel of subject matter experts who evaluate each exam question and then estimate the probability that a minimally competent examinee would answer each test item correctly. The average of the experts' predictions for an exam question becomes its predicted difficulty. The average of the predicted difficulty values across all items on a test is the recommended cut-off point. This point indicates the minimum level of knowledge and skill required to pass.

2.7. Exam Administration

The EHPLE is administered following established protocols and guidelines. Proper exam administration procedures, appropriate security measures, and appropriate consideration for test-takers who need special support will be applied during exam administration at exam centers. The exam is administered in selected HEIs nationally, where candidates can choose based on their convenience at the time of registration. The exam schedule will be posted ahead of time on the MoH website and social media pages. Examinees who have a valid admission paper are eligible to sit for the exam. The mode of exam administration is computer-based testing.

CAUTIONS

-
- Candidates are allowed



- Attend the orientation session in order to sit for the exam
 - Arrive at the exam center on time
 - Bring a legal ID and admission paper
 - Complete the exam within the allotted time frame
-

➤ Candidates are **NOT** allowed



- To bring reference materials, blank paper, or notes into the exam center
- To smoke, eat, or drink in the exam room
- To bring mobile phones, tablets, smart watches, camera devices, eyeglasses, calculators or any type of electronic device into the exam center
- To bring their personal belongings to the exam center
- To bring weapons and sharp materials into the exam center
- To give or receive assistance to or from other candidates during the examination

2.8. Scoring and post exam analysis

Once the exam is completed, the scoring process begins. The exam scoring process involves computerized scoring using software.

Post-exam analysis is the process of analyzing examinees' responses to individual test items in order to assess the quality of the items and the exam as a whole. This phase helps to identify any poorly performing items that may need revision or removal from the exam. The item difficulty index, discrimination index, and reliability coefficient are elements of exam analysis.

2.9. Result notification and appeal management

After scoring and analysis, individual score reports are generated and provided to examinees through the website <http://hple.moh.gov.et>. After result notification, examinees can submit their appeal through phone or email within 10 working days after result notification.

2.10. Licensing

The list of examinees who passed the exam will be sent to regional and city administration regulatory bodies. A license is obtained from the regional/zonal health bodies where he/she permanently lives.

Requirements for professional licensing are:



- Passing the EHPLE
- Original or temporary degree
- Educational documents (10th and 12th certificates)
- Medical certificate
- Government issued ID
- Additional prerequisites based on the requirements of regional regulatory bodies

3. Exam Framework

The key broader professional roles, also known as domains or main knowledge areas serve as a building framework for the licensing examination content for Public Health professional. The domains are further divided into discrete professional attributes that constitute sub-units (also referred to as sub-domains) defining the professional identity of Public Health profession Tasks specifying the performance level of each sub-domain serve as the final characteristic of the professional duties on which the licensing exam focuses.

The contents of the licensing examination are presented below, structured into key roles (domains), sub-units (sub-domains), and tasks. The examination emphasis for each domain and sub-domain, out of the total 100% questions, is indicated in brackets.

Key professional roles/ domains

- ☐ Patient Care (72.0%)
- ☐ Scholar (8.0%)
- ☐ Professionalism (4.0%)
- ☐ Leadership and management (6.0%)
- ☐ Health promotion and disease prevention (10.0%)

Key role/ domain 1: Patient care roles/ domains (62.0%)

Description: This domain encompasses the professional roles of Public Health professional in the provision of high-quality, safe, and patient-centered care within their scope of practice. The provision of up-to-date, ethical, and resource-efficient care requires the application of integrated knowledge of biomedical, clinical, behavioral, and social sciences. As patient care providers, Public Health Professionals shall collect and interpret information, make clinical decisions, carry out diagnostic and therapeutic interventions, and evaluate interventions. To demonstrate competence in this domain, candidates shall apply such integrated knowledge in the following sub-areas:

- ☐ Internal Medicine (18.0%)
- ☐ Surgery (11.0%)
- ☐ Pediatrics and Child Health (15.5%)
- ☐ Gynecology and Obstetrics (14.5%)
- ☐ Dentistry (2.0%)
- ☐ Ear, Nose, and Throat (3.0%)
- ☐ Psychiatry and Mental Health (2.0%)
- ☐ Ophthalmology (3.0%)
- ☐ Dermatology (3.0%)

Key role/ domain 2: Scholar (8.0%)

Description: This domain encompasses the professional roles of Public Health profession in generating and utilizing scientific data to improve the health and well-being of Ethiopians and broaden their scientific knowledge within the healthcare system and community setting. Providing this service requires the application of integrated knowledge in research methods, measurements of health and disease, biostatistics, epidemiology, clinical audit, evidence-based practice, and research ethics. To demonstrate competence in this domain, candidates must possess applied knowledge in planning, problem identification, data collection, analysis, interpretation, report write-up, and dissemination of research outputs.

Key role/ domain 3: Professionalism (6%)

Description: This domain encompasses the professional commitment of Public Health professional to promoting the health and well-being of individuals and society through adhering to ethical standards, maintaining personal integrity, and upholding high standards of competence in all areas of practice. To exhibit competence in this domain, candidates must possess applied knowledge of ethical principles, medicolegal practices, effective communication, accountability to the profession and society, maintenance of professional excellence and personal health, and professional values such as compassion, respect, integrity, honesty, altruism, and humility. To demonstrate competence in this domain candidate shall possess essential applied knowledge of the following sub-areas.

Key role/ domain 4: Leadership and management (6.0%)

Description: This domain encompasses the professional roles of Public Health professional in envisioning a high-quality healthcare system through self-awareness, active participation in healthcare teams, leading teams, and managing health systems. Providing this service requires the application of integrated knowledge in continuous quality improvement, effective health system leadership, management, and healthcare ethics. To demonstrate competence in this domain candidate shall possess applied knowledge to plan, organize, staff, lead, execute, monitor, and control healthcare resources and activities.

Key role/ domain 5: Health promotion and disease prevention (10.0%)

Description: This domain encompasses the professional roles of Public Health professional in enhancing the health and well-being of patients, communities, and the larger populations they serve through health advocacy, disease prevention, health promotion and the promotion of health equity. Providing this service takes an integrated understanding of determinants of health, health informatics, epidemiology, communicable disease control, and health education. To demonstrate competence in this domain candidate shall possess essential applied knowledge of the following sub-areas.

Table 1: Exam Content For Public Health Profession

Domain: Patient Care		
Subdomain: Internal Medicine		
Content	Process	Task
Infectious diseases	Diagnosis	Diagnose TB and HIV
		Diagnose common infectious diseases
	Management	Manage TB and HIV
		Manage common infectious diseases
Gastrointestinal system	Diagnosis	Diagnose peptic ulcer disease and its complications
	Management	Diagnose acute hepatitis, CLD and its complication
Respiratory system	Diagnosis	Manage dyspepsia, peptic ulcer disease and viral hepatitis
	Management	Diagnose respiratory infections, Asthma and COPD
Cardiovascular system	Diagnosis	Diagnose respiratory infections, Asthma and COPD
		Diagnose heart failure, rheumatic fever and RVHD
	Management	Diagnose Hypertension and Ischemic heart disease
		Manage heart failure, Hypertension, Ischemic heart disease and RVHD
Nephrology	Diagnosis	Diagnose nephritic syndrome, nephrotic syndrome CKD and UTIs
	Management	Manage UTIs
Neurology	Diagnosis	Diagnose CNS infection, Cerebrovascular accident seizure disorder and headache
	Management	Manage bacterial meningitis, Cerebrovascular accident and seizure disorders
Endocrinology	Diagnosis	Diagnose DM, Hyperthyroidism and Hypothyroidism
	Management	Identify management principles of DM and its complications
Hematology	Diagnosis	Diagnose anemia
	Management	Manage anemia
Emergency medicine	Diagnosis	Diagnose shock and Poisoning
	Management	Manage shock and poisoning
Subdomain: Surgery		
Content	Process	Tasks
General surgery	Diagnosis	Diagnose type of wound and surgical infections
	Management	Diagnose different types of Goiter, breast lump and breast infections
GI Surgery	Diagnosis	Manage wound, common surgical infections and Benign breast conditions
		Diagnose acute abdomen
	Management	Diagnose GI malignancies, hernias and Perianal conditions
		Manage acute abdomen
Urologic surgery	Diagnosis	Manage patients with perianal conditions
	Management	Diagnose Urinary tract stones, urinary retention, Hydrocele, hematuria and urologic emergencies
Neurologic surgery	Diagnosis	Manage acute urinary retention and urethral injury
	Management	Diagnose Head injury and raised ICP
Orthopedics & Traumatology	Diagnosis	Diagnose bone and joint infections (osteomyelitis, septic arthritis)
		Diagnose fracture, dislocation and compartment syndrome
	Management	Manage bone and joint infections (osteomyelitis, septic arthritis)
		Manage fracture, dislocation and compartment syndrome and polytrauma patient
Cardiothoracic surgery	Diagnosis	Diagnose Chest injury (Hemothorax, Rib fracture, Pneumothorax)
	Management	Manage Chest injury (Hemothorax, Pneumothorax, Rib fracture)
Plastic surgery	Diagnosis	Diagnose burn injury and its complications
	Management	Manage burn injury
Pediatrics surgery	Diagnosis	Diagnose common pediatric surgical conditions

Subdomain: GYN/OBS		
Content	Process	Task
Normal pregnancy	Diagnosis and Follow-up	Diagnose normal pregnancy and Providing ANC follow up
Early and Late pregnancy complications	Diagnosis	Diagnose early pregnancy complication
	Management	Diagnose late pregnancy complication
Normal and Abnormal labor	Diagnosis	Manage early and Late pregnancy complication
	Management	Diagnose normal and abnormal labor
Normal and abnormal puerperium	Diagnosis	Manage normal and abnormal labor
	Management	Diagnose Normal and abnormal puerperium
Menstrual cycle and its abnormalities	Diagnosis	Manage Normal and abnormal puerperium
	Management	Diagnose menstrual cycle and its abnormalities
Gynecologic infections	Diagnosis	Diagnose Gynecological infectious
	Management	Manage gynecological infectious
Family Planning	Diagnosis	Provide Family planning service
Gynaecologic Disorder, Pelvic organ prolapse and Urinary incontinence	Diagnosis	Diagnose Gynecological Disorder, Pelvic organ prolapse and Urinary incontinence
	Prevention	Identify Prevention strategy of Cervical Cancer
Subdomain: Pediatrics and Child Health		
Content	Process	Task
Neonatology	Diagnosis	Diagnose Common Neonatal Disease Conditions
	Manage	Manage Common Neonatal Disease Conditions
Preventive Pediatrics	Diagnosis	Provide immunization service and Identify vaccine preventable disease
	Management	Manage vaccine preventable disease
Infectious diseases	Diagnose	Diagnose Diarrheal Diseases and UTI
	Management	Manage Diarrheal Diseases and UTI
	Diagnosis	Diagnose Tuberculosis and HIV/AIDS
	Management	Manage Tuberculosis and HIV/AIDS
	Diagnosis	Diagnose AFI
	Management	Manage AFI
Gastrointestinal disorders	Diagnosis	Diagnose Protozoal diseases/Helminthes and Acute hepatitis
	Management	Manage Protozoal diseases/helminthes
Cardiology	Diagnosis	Diagnose Cardiac diseases
	Management	Manage Cardiac diseases
Respiratory disease	Diagnosis	Diagnose Community acquired pneumonia, Wheezing, and bronchiolitis
	Management	Manage Community acquired pneumonia, Wheezing, bronchiolitis and bronchitis
Nephrology	Diagnosis	Diagnose Nephrotic/Nephritic syndrome and AKI
Neurology	Diagnosis	Diagnose Seizure disorders and CNS infections
	Management	Manage Seizure disorders and CNS infections
Hematology	Diagnosis	Diagnose common hematologic disorders
Malnutrition	Diagnosis	Diagnose malnutrition
	Management	Manage malnutrition
Growth and development	Diagnosis	Diagnose growth and development
Pediatrics emergencies	Diagnosis	Diagnose Pediatrics emergencies
	Management	Manage pediatrics emergencies
Endocrinology	Diagnosis	Diagnose Diabetes and its complications
	Management	Manage Diabetes and its complications
Subdomain: ENT		
Content	Process	Tasks

Disease of Ear and Nose	Diagnosis	Diagnose Otitis media, Otitis Externa and acute rhino sinusitis
	Management	Manage Otitis externa, AOM, acute rhino sinusitis, and Epistaxis Manage foreign bodies
Pharynx and Salivary gland disorders	Diagnosis	Diagnose tonsillitis & its complications, ATH and deep neck space infection
	Management	Manage tonsillitis
Subdomain: Dentistry		
Tasks		
Diagnose common dental diseases (dental caries, Pulpitis, Gingivitis, periodontitis and dental trauma)		
Manage common dental diseases (dental caries, Pulpitis, Gingivitis, periodontitis and dental trauma)		
Subdomain: Psychiatry		
Task		
Diagnose common psychiatric disorders		
Sub domain: Ophthalmology		
Contents	Process	Tasks
External Eye diseases and red eye diseases	Diagnosis	Diagnose Orbital cellulitis and Conjunctivitis
	Management	Manage Orbital cellulitis and Conjunctivitis
Community Ophthalmology and Eye Injury	Diagnosis	Diagnose Cataract, Trachoma and Vitamin A deficiency
	Management	Manage Trachoma, Vitamin A Deficiency and eye injuries
Subdomain: Dermatology		
Contents	Process	Tasks
Bacterial and viral infection of the skin	Diagnosis	Diagnose bacterial and viral skin infections
	Management	Manage common bacterial and viral skin infections
Superficial Fungal and Parasitic Infections of the Skin	Diagnosis	Diagnose common fungal skin and nail infections and parasitic skin infections
	Management	Manage common fungal skin and nail infections and parasitic skin infections
Eczemas and Acne	Diagnosis	Diagnose atopic skin disorders and acne vulgaris
	Management	Manage atopic skin disorders and acne vulgaris
Domain: Health promotion and disease prevention		
Tasks		
Describe nutritional assessment and intervention methods		
Describe health promotion strategies		
Disaster preparedness		
Outline the Environmental health assessment and intervention		
Assessment of child and maternal health		
Public Health Screening and Surveillance		
Investigate and manage the outbreak		
Domain: Scholar		
Tasks		
Identify the research problem and Setting objective		
Identify the study design, Sampling method, and determine the sample size		
Describe data collection method, tools, data quality assurance, and Ethical consideration		
Identify appropriate data summarization and presentation techniques		
Measure morbidity and mortality		
Measure of association and hypothesis testing		

Domain: Leadership and management
Tasks
Describe Concepts and principles of management
Identify management functions
Resource management
PHC and Ethiopian health policy
Domain: Professionalism
Tasks
Adhere to legal and ethical principles
Establish and maintain a caring, respectful, and collaborative working relationship with clients, client families, the dental team, and other working colleagues
Produce and maintain documentation of procedures and results



Sample questions

1. A 45-year-old female patient comes to a health center with a complaint of productive cough with scanty whitish sputum of one-month duration. She claimed that the cough exacerbates during night time and gets complete relief when she uses Salbutamol puff. She also has chest tightness and pain, and shortness of breath. Her pulse rate is 92/min, respiration rate are 24/min, and temperature is 36.40C. Respiratory examination shows that there is scattered wheeze all over the chest.

What is the most likely clinical diagnosis of this patient?

- | | |
|------------------|----------------|
| (A) Pneumonia | (C) Bronchitis |
| (B) Tuberculosis | (D) Asthma |

Answer Key: The answer is **D**

Explanation: The patient's symptoms of chest tightness and pain, nocturnal cough which relieves with Salbutamol use, and the presence of wheeze are suggestive of asthma. The characteristic symptoms of asthma are wheezing, dyspnea, and coughing, which are variable, both spontaneously and with therapy. Symptoms may be worse at night and patients typically awake in the early morning hours. Typical physical signs are inspiratory, and to a greater extent expiratory wheeze throughout the chest. The diagnosis of asthma is usually apparent from the symptoms of variable and intermittent airways obstruction, but must be confirmed by objective measurements of lung function. On the other hand, Pneumonia (option A), Tuberculosis (option B), and Bronchitis (option C), cannot be the most likely clinical diagnosis in this case as they have different symptoms.

2. A 31-year-old male patient comes to a clinic with complaints of fever and abdominal pain of five days duration. The fever has a characteristic of stepladder increment. He also has headache, joint pain and anorexia. His pulse rate is 64/min, temperature is 39.60C, blood pressure is 120/70mmHg and respirations rate are 20/min. On examination, there is no remarkable systemic finding. His CBC result shows WBC count of 2500/mm³, and hematocrit level of 43%.

What is the most likely diagnosis of the patient?

- | | |
|---------------------|--------------------------|
| (A) Typhoid fever | (C) Cerebral malaria |
| (B) Relapsing fever | (D) Bacterial meningitis |

Answer Key: The answer is **A**

Explanation: The patient's symptoms of high grade fever with stepladder increment, abdominal and joint pain, headache, anorexia, and presence of relative bradycardia and leucopenia are suggestive of typhoid fever (enteric fever), which is a systemic disease caused by dissemination of *S. Typhi* or *S. Paratyphi*.

3. A 28-year-old female comes to an OPD with recurrent left side headache of three-day duration. She had similar history of headache for most of the days of the month for the last two years. The pain worsens when she works over night at duty hours. She also has nausea, vomiting and fear of light accompanying the headache. Her mother has also similar complaints. Her vital signs are in the normal range. There is no pertinent physical finding on examination

What is the most likely diagnosis of the patient?

- | | |
|----------------------|------------------------------|
| (A) Cluster headache | (C) Traction headache |
| (B) Tension headache | (D) Migraine headache |

Answer Key: The answer is **D**

Explanation: The patient's symptoms, recurrent left sided headache which accompanied by nausea, vomiting and fear of light, the presence of similar illness for the past two years, and the presence of family history are more suggestive of migraine headache unlike the other primary headaches. The other options (A, B. & C) cannot be the most likely diagnosis of the patient in this case. In the case of tension and traction headaches (options B & C), the headache is bilateral and do not accompany by nausea, vomiting or fear of light against their diagnosis. In the case of cluster headache (option A), the pain is retro-orbital and characterized by its periodicity.

4. A 52-year-old female, who was working standing seven-hour per-day for the last three-years, comes to a clinic with a complaint of leg pain of one-week duration. She also has aching and heaviness over both legs. On musculoskeletal examination, there is bluish colored, multiple tiny threads like distended veins over both legs.

What is the most appropriate initial management for the patient?

- | | |
|-------------------------------|-------------------------------|
| (A) Compression stocking | (C) Surgical venous stripping |
| (B) Multiple venous ligations | (D) Endovenous laser ablation |

Answer Key: the answer is **A**

Explanation: From the history, the patient was working with prolonged standing, leg pain; aching and heaviness of both legs and the presences of tiny threads like dilated veins and bluish discoloration suggest the diagnosis of varicose vein. The most appropriate initial management is compressive elastic stocking or bandage. For this patient, the history and physical examination findings do not suggest the need of surgical intervention at this stage.

5. A 28-year-old known RVI male patient on HAART comes to a clinic with a complaint of a painful swelling around his anus of four days duration. He also has fever, chills and rigor. On examination, there is 3x2cm sized very tender, hot to touch and smooth surfaced mass on lateral side of anal verge. When it is tapped, frank pus is aspirated.

What is the most appropriate management for the patient?

- (A) Incise and drain the pus, and then put on broad spectrum antibiotics
- (B) Put him on oral broad spectrum antibiotics and send him home
- (C) Needle aspiration and oral broad spectrum antibiotics
- (D) Give potent anti-pain and send him home

Answer Key: The answer is A

Explanation: The patient's symptoms, painful swelling around the anus and the presence of tender, hot to touch mass on the lateral side of the anal verge and the tap revealing frank pus are suggestive of perianal abscess. The appropriate management for perianal abscess is incision and drainage of the abscess to prevent severe necrotizing infections. Antibiotics would not be recommended in all perianal abscesses and in most of the cases incision and drainage would suffice. But, for this patient the presence of immunocompromisation (known RVI on HAART) and systemic manifestations (fever, chills and rigor) indicate the need for initiation of broad spectrum antibiotics in addition to incision and drainage of the abscess. Therefore, the most appropriate management for the patient is incising, draining, and then putting on broad spectrum antibiotics.

6. A three-month-old male infant come with his mother to a health center with poor feeding, failure to thrive and sweating while feeding. Physical examination is remarkable for cyanosis, tachypnea, tachycardia, a gallop rhythm and hepatomegally.

What is the most appropriate initial management of the infant?

- (A) Put on oxygen and refer
- (B) Put on antibiotic and refer
- (C) Start diuretics and send home
- (D) Start antibiotics and send home

Answer Key: The answer is A

Explanation: From the symptoms and physical findings, the age of the infant, the presence of poor feeding, failure to thrive & sweating while feeding, cyanosis, tachycardia, tachypnea, gallop and hepatomegally are suggestive of cyanotic heart disease with respiratory distress. The most appropriate initial management is support the infant with oxygen until he gets definitive managements. The definitive managements will be medical or surgical. Therefore, the most appropriate immediate step for the infant at the health center level is to put on oxygen and refer for the definitive management.

7. A two-year-old child is presented with cough of one-day duration and worsening of respiratory difficulty. The family reported that the child was playing with other children in the compound before the incident and then had choking episode. On examination, vital signs are in the normal range and wheezing on the right lung field.

What is the most appropriate initial step in the management of this case?

- | | |
|--------------------------|-----------------|
| (A) Foreign body removal | (C) Antibiotics |
| (B) Intranasal oxygen | (D) Steroids |

Answer Key: The answer is **A**

Explanation: From the symptoms and the physical findings, the presence of cough, worsening of respiratory difficulty, choking episodes and localized wheeze on the right lung field supports the diagnosis of foreign body aspiration than other possible causes of wheezing. The removal of the foreign body is the most appropriate management to prevent the subsequent complications associated with foreign body aspiration. Intranasal oxygen can be the answer but the vital signs are in the normal range and the child is not in need of intranasal oxygen support. Antibiotics are not recommended in a child with foreign body aspiration unless the foreign body results localized infections and there are no symptoms. Steroids are mostly used for the management of croup (laryngotracheobronchitis) after the child is presented with upper airway obstructions than for foreign body aspiration.

8. A 16-year-old female student was brought by her mother to the gynecologic OPD with a complaint of pain during menses of one-year duration. The pain is severe and subsequently she missed school many times. She claimed that she visited different health facilities for her complaint but no problem was identified. Physical examination and investigations were all unremarkable.

What is the most likely diagnosis for the patient?

- | | |
|--------------------------|----------------------------|
| (A) Endometriosis | (C) Secondary dysmenorrhea |
| (B) Primary dysmenorrhea | (D) Membranous amenorrhea |

Answer Key: The answer is **B**

Explanation: From the history the client's age, the presence of cyclic pain during menses & the absence of any underlying problem and the unremarkable findings from the physical and investigations support the diagnosis of primary dysmenorrhea. The clinical presentation of primary and secondary dysmenorrhea will be similar but usually clients with secondary dysmenorrhea have underlying pathology such as endometriosis, adenomyosis, PID that results the symptoms of secondary dysmenorrhea. Amenorrhea is the absence of menses but the client already has menses, hence membranous amenorrhea cannot be the answer

9. A 22-year-old G-II P-I woman presented to a labor ward at 38 weeks of gestation with gush of fluid per vagina of two hours duration. She has associated lower abdominal pain that comes every three minutes. Her blood pressure was 100/70mmHg, pulse rate was 86/min and temperature was 37°C. FHB was 144/min. Pelvic examination revealed cervical dilatation of 3cm and station of -1 and adequate pelvis. Her laboratory results were within a normal range.

What is the most appropriate management for this woman of the lady?

- | | |
|--------------------------------|-------------------------------|
| (A) Admit her and follow labor | (C) Encourage her to ambulate |
| (B) Augment her with Oxytocin | (D) Start her on Antibiotics |

Answer Key: The answer is A

Explanation: From the history, the woman is pregnant at her 38 weeks of GA presented with gush of fluid per vagina and associated lower abdominal pain, which comes every 3 minutes, indicating the rupture of the membrane and beginning of labor. As well, on physical examination the cervical dilation of 3cm and station of -1 reveals she is in latent first stage of labor. The presence of maternal and fetal normal vital signs and the adequacy of the pelvis show the labor is normal. The most appropriate management is admitting her and follows labor.

10. A 40-year-old Gravida VI Para V woman, whose gestational age is 39 weeks, is presented to health center with complaints of lower abdominal and back pain of six hours duration. On physical examination, vital signs are in the normal range, had 3 mild contractions and FHB was 145/min. On per-vaginal examination, cervix was 3 cm dilated, position was left occiput-transverse, station was -1 and membrane was intact.

What is the most likely diagnosis of this woman?

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|---------------------------|---------------------------------|
| (A) Third stage of labor | (C) Active first stage of labor |
| (B) Second stage of labor | (D) Latent first stage of labor |

Answer Key: The answer is D

Explanation: From the history and physical examination, the lower abdominal & back pain of six hours and the presence of 3 mild (lasts for 20-40 seconds) uterine contractions, indicate the onset of labor. The per-vaginal examination also reveals cervical dilatation of 3 cm which is latent first stage of labor.

11. A nine-month-old infant comes with his mother for a complaint of cough and fast breathing of one-week duration. The infant has no previous immunization history. After a thorough evaluation, he was diagnosed with pneumonia.

What is the most appropriate intervention to prevent recurrence of the disease?

- (A) Zinc supplementation (C) TT immunization
(B) Measles vaccination (D) Antibiotics

Answer Key: The answer is **B**

Explanation: - From the history, the child is nine-months-old with cough and fast breathing supports the diagnosis of pneumonia. The child also unvaccinated or no history of immunization since birth, which will contribute for development of pneumonia or recurrent pneumonia. According to the expanded program for immunization / EPI/, most of the vaccines used for pneumonia prevention directly or indirectly. Out of the ten vaccines, which are given for children according to the Ethiopian EPI schedule, BCG, pneumococcal conjugated vaccine (PCV), pertussis, diphtheria and measles vaccines contribute for the prevention of pneumonia. Therefore, the child unvaccinated for measles has a high risk for recurrent disease or pneumonia.

12. A farmer working at the edge of a river experienced repeated insect bite by insects coming from the river. Two weeks later, he became febrile and started developing papules, which then changed to ulcer.

What is the most likely vector that transmitted this disease?

- (A) Black fly (B) Sand fly (C) Flesh fly (D) Fruit fly

Answer Key: The answer is **B**

Explanation: From the history, the farmer is working at the edge of the river, which is one of the breeding sites for sand flies. The presence of repeated insect bite and the development of fever and papular lesions which later change to ulcer favors the diagnosis of cutaneous leishmaniasis, which is transmitted by the bite of a vector sand fly.

13. While performing physical examination, a health officer found small tumor on the patient's left lower extremity. The procedure was on the next keeping exact order of patient flow. On the date of operation, the health officer did incision and removes the tumor without patients consent.

What ethical principle did the health officer violated?

- | | |
|---------------------|--------------|
| (A) Non-Maleficence | (C) Autonomy |
| (B) Beneficence | (D) Justice |

Answer Key: The answer is C

Explanation: Respect for autonomy is one ethical principle where clients exercise their own self-determination. In the scenario the client was not given a choice to choose between treatment options and the procedure done without consent. Therefore, this is a clear breach of patient autonomy. On the other hand, non-maleficence (Choice A) is an act of avoiding harm was not discussed in the scenario directly. In the same manner, Beneficence (choice B), which is also an act of doing for benefit of patient was not discussed directly in the scenario. Again, justice (choice D) is not violated since all patients were treated in their order of arrival.

14. A manager at a district health office delegate's position for different departments under his scope of management, and he applied his own leadership style for the success of his organization. For this, he maintains strong control over people within each department and focuses on tasks rather than people and he makes all the decisions by himself and communicates the decision for his subordinates.

What is the most appropriate leadership style followed by a manager in this case?

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|-------------------|-----------------|
| (A) Participatory | (C) Leizas-fair |
| (B) Democratic | (D) Autocratic |

Answer Key: The answer is D

Explanation: The autocratic leader maintains strong control over people in the group. This control may be paternalistic leadership or it may be dictatorial, with the complete disregard for the needs and feelings of group members. Authoritarian leaders unlike participatory and democratic leaders give orders and expect group members to obey these orders. Directions are given as commands, not suggestions. This approach generally results in passive resistance from team members and requires continual pressure and direction from the leader in order to get things done.

15. A spontaneous rupture of unscarred uterus in a primigravid patient is extremely rare clinical condition. However, a 34-year-old primigravid woman came to an emergency OPD with sudden acute abdomen. An emergency laparotomy was performed and a uterine rupture was found as the cause of the event.

What is the most appropriate study design for this scenario?

- | | |
|-----------------|---------------------|
| (A) Case series | (C) Case control |
| (B) Case report | (D) Cross sectional |

Answer Key: The answer is **B**

Explanation: Case report is detail reporting of unusual/rare events on the single case by one or more physicians. In the above scenario we have the information that indicates unscarred uterus in a primigravid patient which is extremely rare clinical condition and observed on this woman. Hence, case report is the best answer for the scenario. But, choice A is detail reporting on a single case with number (5-12) of individuals, choice C is for identifying the risk factors of disease from previous exposure status, and choice D is a method of studying the prevalence of diseases.

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List of contributors

Mr. Endalkachew Tsedal	MoH/ HHRIPR LEO
Dr. Meron Yakob	MoH/ HHRIPR Exam Development and Administration Desk Lead
Ermias G/Yohannes	MoH/ HHRIPR
Dr. Wudasio Teshome	MoH/ HHRIPR
Dr. Melese Achamo	MoH/ HHRIPR
Tsedale Tafesse	MoH/ HHRIPR
Mekonnen Desie	MoH/ HHRIPR
Abera Bezabih	MoH/ HHRIPR
Fikadie Dagnaw	MoH/ HHRIPR
Melkamu Gereme	MoH/ HHRIPR
Tadele Asefa	MoH/ HHRIPR
Neway H/Michael	MoH/ HHRIPR
Mebrahtu Tarekegn	MoH/ HHRIPR
Milkoman Ayele	MoH/ HHRIPR
Bethelhem Shikabew	MoH/ HHRIPR
Amanuel Tesfay	MoH/ HHRIPR
Dr. Alemseged Woretaw	Jhpiego/HWIP
Dr. Yohannes Molla	Jhpiego/HWIP
Bekalu Assamnew	AMREF HWIP project
Dr. Equinet Misganaw	AMREF HWIP project



Dr Belay Hagos	AAU- IER
Dr Birhanu Abera	AAU- IER
Dr Girma Lema	AAU- IER
Dr Daneal Desta	AAU- IER
Dr Firdisa jebessa	AAU- IER
Dr Wessenu Yimam	AAU- IER
Dr. Abdnasir Ahmed	AAU- IER
Tamirat Tekilu	Ethiopian Health Students Association
Zelalem Negash	Yekatit 12 Hospital and Medical Collage
Wassie Negash	Debre Birhan University
Sisay Shunie	Debre Birhan University
Agezegn Asegeid	Wachemo University

Contact Us

Ministry of Health – Ethiopia

Street Address: - 1234 Sudan Street, Addis Ababa – Ethiopia

Telephone: +251-118275936

Hotline service: - 952

Website: <https://www.moh.gov.et>

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